

**STATE OF VERMONT  
SECRETARY OF STATE  
OFFICE OF PROFESSIONAL REGULATION  
BOARD OF ALLIED MENTAL HEALTH PRACTITIONERS**

In re: ALISON M. GRANT  
License No. 068.0057635

}  
}  
} Docket No. 2016-99  
}

**DECISION ON REQUEST FOR  
SUMMARY SUSPENSION ORDER**

Members participating:

William Scarlett, Vice-Chair  
Joshua Stumpff, *ad hoc*  
Scott Giles, Public Member  
Michael Doyle, *ad hoc*, Public Member

Appearances:

Prosecuting the case: Rachel Allen, Esq.  
Respondent: the Respondent did not appear, nor was she represented

Presiding Officer: George K. Belcher

Exhibits:

State Exhibit 1: Summary of Billing Data

**FINDINGS OF FACT, CONCLUSIONS OF LAW,  
AND ORDER**

This matter came before the Board of Allied Mental Health Practitioners on a Request for a Summary Suspension. The hearing was held on August 18, 2016 at the Office of Professional Regulation in Montpelier, Vermont. The Respondent was not present and was not represented. The State was represented by Rachel Allen, Esq. The Board has authority to summarily suspend a license pending further action, if it determines that public health, safety, or welfare imperatively requires emergency action. 3 V.S.A. § 814(c).

**Findings of Fact**

- (1) Based on a review of the pleadings, and upon the evidence presented at the hearing, the Board finds a follows:
- (2) Respondent is a Licensed Clinical Mental Health Counselor and is therefore subject to the regulatory authority of this Board. 3 V.S.A. §§ 129, 129a, 814(c), and 26 V.S.A. Chapter 65, the Administrative Rules of the Board of Allied Mental Health Practitioners, and the Rules of the Office of Professional Regulation. 3 V.S.A. § 814(c) permits the

Board to summarily suspend a license if it finds that public health, safety, or welfare imperatively requires emergency action, and incorporates a finding to that effect in its order. The Respondent was originally licensed on December 17, 2009 and her license is currently set to expire on January 31, 2017.

- (3) The prosecutor filed a "Request for Summary Suspension" dated August 8, 2016. The Request alleges that the Respondent: failed to comply with provisions of federal or state statutes or rules governing the practice of the profession (3VSA Sec. 129a(a)(3)); performed conduct which evidences unfitness to practice clinical mental health counseling (26 VSA Sec. 3271(a)(3)); failed to retain client records for a period of seven years (3 VSA Sec. 129a(a)(9)); and entered into an additional relationship with a client that might impair her objectivity or otherwise interfere with her professional obligations (26 VSA Sec. 3271(a)(6)). The Request for Summary Suspension further alleges that the public health, safety or welfare imperatively requires emergency summary suspension of the license of the Respondent.
- (4) Notice of this hearing was mailed to the Respondent on August 9, 2016 by ordinary and certified mail. There was no evidence that the mailings were received or not received. The certified mail return receipt was not yet returned as of the date of the hearing and the ordinary mail notice was not returned. The State prosecutor had no contact with the Respondent and there was no indication from her that she intended to participate in the hearing.
- (5) The State introduced the testimony of John Lewis who is an investigator with the Office of Professional Regulation. Mr. Lewis testified that he interviewed the Respondent on July 19, 2016 in her living room, which also functioned as her office. On July 19, 2016 as Mr. Lewis was entering the Respondent's "office" a client was leaving. The Respondent admitted that she had received an Adderall tablet from this client and that she had already consumed it.
- (6) Mr. Lewis first described how the Respondent seemed to have a hard time distinguishing between her clients and her friends. One of her clients first approached the Respondent by asking her for a car ride and riding around with her for a while. This occurred on June 17, 2015 but, despite having no contact with the Respondent after June of 2015, the Respondent billed for treatments of the client during July through October of 2015.
- (7) At least two clients of the Respondent lived with her for periods of each about one week while they were in treatment with her. The Respondent admitted to Mr. Lewis that she loaned money to clients and that several of her clients owed money to her which they borrowed.
- (8) The Respondent is alleged to have taken a client to a bar during a treatment session. (See allegation number 32).
- (9) When the Respondent was having difficulty responding to inquiries from the Vermont Attorney General's Office about her records, she asked some of her clients to help her reconstruct her records.
- (10) Some of the Respondent's clients complained about breach of confidence between the Respondent and other clients/friends.
- (11) The Respondent has had a valid prescription for Percocet. The Respondent admitted to Mr. Lewis that she would trade Percocet tablets to some of her clients in

---

<sup>1</sup> At the hearing the State made an oral motion to paragraph 24, line 5 of the Request for Summary Suspension to read "R.S. did not receive any services after [2013] 2015". The motion was granted.

exchange for Adderall. She used Adderall without a prescription because of the way that it made her feel. Percocet is a regulated drug containing acetaminophen and oxycodone. Adderall is a regulated drug which contains amphetamine.

- (12) The Respondent admitted to Mr. Lewis that she and another person would travel to a place on a highway and there buy Percocet tablets without a prescription from an unspecified dealer.
- (13) When asked by Mr. Lewis whether the Respondent had a drug addiction, she initially denied it but later stated that she was unsure whether she was addicted.
- (14) The Respondent admitted to Mr. Lewis that she was having problems keeping her client records. In one file the only record was a copy of the client's Medicaid card despite approximately three years of treatment. In another file, the last entered note or record was in 2014 and yet the client had received service throughout 2015 according to her billings.
- (15) The Respondent admitted shredding files from 2012-2014 for another client and that she was not aware of the requirement to keep files for seven (7) years. (See Paragraph 43 of Request for Summary Suspension).
- (16) Finally, the Respondent appears to have been fraudulently billing for services not rendered. She used her daughter to prepare and submit bills to her payor. She reported to her daughter emergency services to clients which she had not rendered. She admitted this to Mr. Lewis.
- (17) The allegations contain numerous instances of overbilling and fraudulent billing. (See paragraphs 13-24). Despite having only twelve clients per week, being a sole practitioner, and practicing out of her home, the Respondent was the top biller of crisis code (#90893 and #90840) claims for reimbursement in 2014, 2015, and 2016. In 2015 she represented 37.91% of all Vermont Medicaid reimbursements for code #90839 with 72 providers making up the balance of 62.09 % of the claims for that code. (See State Exhibit 1).
- (18) These allegations, coupled with the admissions to Investigator Lewis, persuasively show the Respondent to be a mental health professional who has lost her bearings surrounding ethical and professional conduct and judgment.

## CONCLUSIONS OF LAW

- (1) The State has the burden of proof to establish that the public health, safety or welfare are at risk and that the proven risk to the public imperatively require emergency action. See 3 VSA Sec. 814(c). The timing, nature, and purpose of the administrative hearing may define the scope and procedural parameters of an administrative hearing. See In re Miller, 2009 VT 112, 989 A.2d 982 (Vt. 2009) at Para. 14. In summary suspension hearings, the administrative authority may look to the allegations, as well as the factual basis for the allegations, to make its findings.
- (2) It is possible for a board to make a summary suspension findings based, in part, upon mere allegations, if the allegations are so severe as to establish an emergency to which notice, delay, and pre-deprivation hearing are not justified, and if a further administrative review is promptly available. See In re Miller, *supra*. citing Alger v. Dept. of Labor, 2006 VT 115, 181 Vt. 309, 917 A.2d 508. See also, Barry v. Biarchi, 443 U.S. 55, 99 S.Ct. 2642, 61 L.Ed.2d 365 (1979) which held that the state may impose a summary license

suspension "... whenever it has satisfactorily established probable cause to believe [a violation has occurred]" Id. at p.65, emphasis added.

- (3) Evidence of flawed professional judgement may justify a summary suspension if the flawed professional judgment were connected to the "risks that imperatively called for emergency suspension". In re Glenn Myer, Washington Sup. Ct. 140-2-07 Wncv, page 13. Here, the evidence of drug dealing, breach of professional/client separation standards, poor and non-existent client record-keeping, and fraudulent client billing, all expose the Respondent as being adrift from her professional responsibilities. Her judgment has been negatively affected. Moreover, her clients are, and have been, at significant risk. For example, if one of her clients were to decompensate or be psychiatrically hospitalized, the treatment record might be unavailable in the crisis. Her professional objectivity with those clients with whom she is drug-dealing is no doubt compromised.
- (4) If the Respondent has such numbed sensitivity to such basic rules and restrictions upon her mental health practice, her judgment is clearly impaired. Such impaired judgment unnecessarily puts any client who seeks and pays for her *professional* judgment in a risky and dangerous situation.
- (5) In this case the State has adequately established that protection of the public requires summary suspension.
- (6) The Findings of Fact and Conclusions of Law in this Summary Suspension hearing are for purposes of deciding whether *at this time* there is an imperative need to take emergency action. 3 V.S.A. § 814(c). The Findings of Fact and Conclusions of Law herein are for purposes of this Order only.
- (7) At any merits hearing in this matter, the prosecutor will bear the burden of proving unprofessional conduct. The Findings of Fact and Conclusions of Law at a contested hearing in this matter in the future will be based exclusively on the evidence admitted at that hearing. This order is subject to motions for reconsideration but shall be effective immediately upon entry.

### Order

The **Request for Summary Suspension** is **GRANTED**.

Board of Allied Mental Health Practitioners

By: William Scarlett  
William Scarlett, Vice Chair

Date: 8/22/2016

OFFICE OF PROFESSIONAL REGULATION

DATE OF ENTRY: 8/23/16

### **APPEAL RIGHTS**

This is a final administrative determination by the Vermont Board of Allied Mental Health Practitioners. A party aggrieved by a final decision of a board may appeal this decision by filing a written Notice of Appeal with the Director of the Office of Professional Regulation, Vermont Secretary of State, 89 Main Street, Fl. 3, Montpelier, VT 05620-3402 within 30 days of the entry of this order. If an appeal is filed, the Director of the Office of Professional Regulation shall assign the case to an appellate officer. The review shall be conducted on the basis of the record created before the board. In cases of alleged irregularities in procedure before the board, not shown in the record, proof on that issue may be taken by the appellate officer. 3 V.S.A. §§ 129(d) and 130a. To request a stay of the Board's decision, please refer to the attached stay instructions.

**STATE OF VERMONT  
SECRETARY OF STATE  
OFFICE OF PROFESSIONAL REGULATION  
BOARD OF ALLIED MENTAL HEALTH PRACTITIONERS**

**IN RE:**  
**ALISON M. GRANT**  
**License No. 068.0057635**

)  
)  
)

**Docket No. 2016-99**

**REQUEST FOR SUMMARY SUSPENSION**

**Board Authority**

1. The Vermont Board of Allied Mental Health ("Board") has jurisdiction to investigate and adjudicate allegations of unprofessional conduct, to license and renew licenses, pursuant to 3 V.S.A. §§ 129, 129a and 814; 26 V.S.A. § 4083; the Rules of the Board of Allied Mental Health Practitioners and the Rules of the Office of Professional Regulation.
2. The Vermont Board of Allied Mental Health is authorized by 3 V.S.A. § 814 to summarily suspend the license of a licensed clinical mental health counselor when it finds that the public health, safety, or welfare imperatively requires emergency action.

**General Statement of Facts**

3. Alison M. Grant ("Respondent") of Rutland, Vermont, is licensed by the State of Vermont as a Licensed Clinical Mental Health Counselor under license number 068.0057635. This license was originally issued on December 17, 2009, and is currently set to expire on January 31, 2017.
4. At all times relevant, Respondent was self-employed as a Licensed Clinical Mental Health Counselor at her private practice located in Rutland, Vermont.
5. Respondent is a sole practitioner, with approximately twelve clients per week, who practices out of her home office, out in the community, or in the client's home.
6. Respondent traded and sold unprescribed medications and her personal prescription for Percocet with her clients.
7. Respondent traded Percocet in exchange for Adderall with two clients.

STATE OF VERMONT



Prosecuting Attorney  
Office of  
Professional Regulation  
89 Main Street  
3rd Floor  
Montpelier, VT  
05620-3402

8. With the assistance of a client, A.G., Respondent routinely purchased twenty additional Percocet from a family friend of A.G.
9. Respondent sold Percocet pills when she needed money.
10. Respondent has a valid prescription for Percocet 10/325 and Phentermine 37.5 milligrams. In addition to her prescriptions and the Percocet obtained from A.G.'s family friend, Respondent used Adderall without a prescription because of the way it made her feel. Respondent stated to Investigator John Lewis, Investigator with the Office of Professional Regulation, that she was unsure if she had an addiction to these substances.

**Violation One: 3 V.S.A. § 129a(a)(3). Failure to comply with provisions of federal or State statutes or rules governing the practice of the profession, incorporating 33 V.S.A. § 141(d).**

**33 V.S.A. § 141(d). A person who knowingly files, attempts to file, or aids and abets in the filing of a claim for services to a recipient of benefits under a State or federally funded assistance program for services which were not rendered; or who knowingly files a false claim or a claim for unauthorized items or services under such a program; or who knowingly bill the recipient of benefits under such a program or his or her family for an amount in excess of that provided for by law or regulation; or who knowingly fails to credit the State or its agent for payment received from Social Security, insurance, or other sources; or who in any way knowingly receives, attempts to receive, or aids and abets in the receipt of unauthorized payment as provided herein shall be punished as provided in section 143 of this title.**

11. The State re-alleges and incorporates Paragraphs 3 through 5 above.
12. Respondent billed Medicaid falsely to obtain money for personal expenses and drugs.
13. As a Medicaid provider, Respondent billed the State of Vermont for her therapy sessions. "Procedure Code 90839" is the Medicaid billing code for a crisis psychotherapy session and allows the Respondent to collect more money than the standard psychotherapy session code. "Procedure Code 90840" is the Medicaid billing code for an additional 30 minutes of a crisis therapy session and allows the Respondent to collect more money than a standard additional 30 minutes of a psychotherapy session.
14. In 2014, Respondent was the highest biller in Vermont for Code 90839 at a rate of 12.87%, for a title of \$4,777.23.
15. In 2014, Respondent was the highest biller in Vermont for Code 90840 at a rate of 35.27%, for a total of \$2,212.17.

STATE OF VERMONT



Prosecuting Attorney  
Office of  
Professional Regulation  
89 Main Street  
3rd Floor  
Montpelier, VT  
05620-3402

16. In 2015, Respondent was the highest biller in Vermont for Code 90839 at a rate of 37.91%, for a total of \$12,307.44.
17. In 2015, Respondent was the highest biller in Vermont for Code 90840 at a rate of 66.73%, for a total of \$5,937.93.
18. From January 2016 to July 6, 2016, Respondent was the highest biller in Vermont for Code 90839 at a rate of 40.84%, for a total of \$8,395.38.
19. From January 2016 to July 6, 2016, Respondent was the highest biller in Vermont for Code 90840 at a rate of 67.96%, for a total of \$3,933.72.
20. On February 26, 2016, Respondent admitted to the following Medicaid billing practices:
  - a. Billing for providing rides to clients;
  - b. Billing for going to a bar with a patient because she was providing "socialization skills"; and
  - c. Billing for sessions at laundromats and in a car.
21. On July 1, 2016, Respondent stated she usually made weekly handwritten lists of clients she saw. Respondent's daughter, a dental hygienist, electronically billed Medicaid based on Respondent's information.
22. On July 1, 2016, Respondent admitted that she had seen a client's granddaughter for services. Because the granddaughter did not yet have a Medicaid number, Respondent billed the grandmother, D.H., for the granddaughter's services.
23. Respondent billed M.P.'s Medicaid for approximately 4 months of services in 2015 that were not scheduled or provided. Respondent saw M.P. only once for an initial assessment on June 17, 2015, after taking M.P. to a doctor's appointment. Respondent continued to bill M.P. for services that were not scheduled or provided. From July 2015 to October 2015, M.P.'s Medicaid was billed for services 23 times, 5 of these sessions were billed as crisis visits, and 5 of these sessions were billed as crisis visits extending beyond the normal session time.
24. Respondent billed R.S.'s Medicaid for approximately 6 months of services in 2016 that were not scheduled or provided. Notes in R.S.'s client file indicated that Respondent began seeing R.S. in July 2011. The last progress note was dated in April 10, 2013. R.S. had sporadic phone calls with Respondent until the end of 2015. R.S. did not receive any services after 2013. From January 2016 to June 2016, R.S.'s Medicaid was billed for services 30 times, 5 of these sessions were

STATE OF VERMONT



Prosecuting Attorney  
Office of  
Professional Regulation  
89 Main Street  
3rd Floor  
Montpelier, VT  
05620-3402

billed as crisis visits, and 3 of these sessions were bill as crisis visits extending beyond the normal session time.

25. As a Licensed Clinical Mental Health Counselor, Respondent must comply with provisions or federal or State statutes or rules governing the practice of the profession.
26. As a Licensed Clinical Mental Health Counselor, Respondent is prohibited from knowingly filing, attempting to file, or aiding and abetting the filing of a claim for services to a recipient of benefits under a State or federally funded assistance program for services which were not rendered.
27. The facts alleged in Paragraphs 12 through 24 demonstrate Respondent's failure to comply with state statute by knowingly filing a claim for services not rendered to Medicaid recipients.
28. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because the Respondent has committed unprofessional conduct in violation of 3 V.S.A. § 129a(a)(3), incorporating 33 V.S.A. § 141(d).

**Violation Two: 26 V.S.A. 3271(a)(6). Entering into an additional relationship with a client, supervisee, research participant or student that might impair the licensed clinical mental health counselor's objectivity or otherwise interfere with the clinical mental health counselor's professional obligations.**

29. The State re-alleges and incorporates Paragraphs 3 through 10 above.
30. While providing professional services to active clients, Respondent engaged in friendships with clients, provided a Thanksgiving dinner for clients, provided rides to clients, accepted rides from clients, provided money to client, and accepted money from clients.
31. Respondent allowed two clients to live with her while active clients. S.W. and her boyfriend lived with Respondent for approximately a week. M.S. lived with Respondent for approximately a month.
32. During a session, Respondent took a client to a bar.
33. Respondent attended a family function for D.H., a minor client.
34. Respondent frequently acted a case manager for her clients when she felt they needed the services of a case manager.
35. As a Licensed Clinical Mental Health Counselor, Respondent is prohibited from entering into an additional relationship with a client that might impair her

STATE OF VERMONT



Prosecuting Attorney  
Office of  
Professional Regulation  
89 Main Street  
3rd Floor  
Montpelier, VT  
05620-3402

objectivity or otherwise interfere with a clinical mental health counselor's professional obligations.

- 36. The facts alleged in Paragraphs 6 through 8 and 30 through 34 demonstrate Respondent's failure to maintain professional boundaries.
- 37. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because the Respondent has committed unprofessional conduct in violation of 26 V.S.A. § 3271(a)(6).

**Violation Three: 3 V.S.A. § 129a(a)(9). Failure to retain client records for a period of seven years, unless laws specific to the profession allow for a shorter retention period. When other laws or agency rules require retention for a longer period of time, the longer retention period shall apply.**

- 38. The State re-alleges and incorporates Paragraphs 3 through 5 above.
- 39. Respondent failed to keep or maintain a client file for M.P. Respondent stated to Investigator Lewis that M.P., "fell through the cracks."
- 40. On July 1, 2016, when asked about a missing client file, Respondent explained that she may have shredded it because she had not seen her in a couple of weeks.
- 41. The client file for D.M., a client of approximately three years, contained only a copy of the client's Medicaid card.
- 42. On or about July 1, 2016, Respondent stated that she kept poor records.
- 43. Respondent stated that she was not aware that she needed to keep client files for seven years. Respondent stated that she shredded client files from 2012, 2013, and 2014.
- 44. As a Licensed Clinical Mental Health Counselor, Respondent must retain client records for a period of seven years.
- 45. The facts alleged in Paragraphs 39 through 43 demonstrate Respondent's failure to keep and retain client records for seven years.
- 46. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because the Respondent has committed unprofessional conduct in violation of 3 V.S.A. § 129a(a)(9).

**Violation Four: 26 V.S.A. § 3271(a)(3). Conduct which evidences unfitness to practice clinical mental health counseling.**

- 47. The State re-alleges and incorporates Paragraphs 3 through 43 above.

STATE OF VERMONT



Prosecuting Attorney  
Office of  
Professional Regulation  
89 Main Street  
3rd Floor  
Montpelier, VT  
05620-3402

48. As a Licensed Clinical Mental Health Counselor, Respondent is prohibited from trading and selling prescription drugs, especially with clients seeking professional services.
49. The facts alleged in Paragraphs 6 through 10 (drug use), 12 through 24 (Medicaid billing), 30 through 34 (professional boundaries), and 39 through 43 (client files) demonstrate Respondent's unfitness to practice clinical mental health counseling.
50. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because the Respondent has committed unprofessional conduct in violation of 26 V.S.A. § 3271(a)(3).

**Request for Relief**

51. Emergency action is required because Respondent's continued licensure is a significant threat to the health, safety and welfare of the people of the State of Vermont because a Licensed Clinical Mental Health Counselor is prohibited from engaging in the selling and trading of prescription medication and required to properly bill, maintain professional boundaries, and keep and maintain proper client records.
52. The facts as set out above establish that in order to protect the public health, safety, or welfare of the people of the State of Vermont, emergency action is imperative.

**WHEREFORE**, the State of Vermont respectfully requests that pursuant to 3 V.S.A. § 814(c), the Respondent's licensed clinical mental health counselor license, number 068.0057635, be summarily suspended, pending a hearing on the merits.

**DATED** at Montpelier, Vermont this 8<sup>th</sup> day of August, 2016.

STATE OF VERMONT



Prosecuting Attorney  
Office of  
Professional Regulation  
89 Main Street  
3rd Floor  
Montpelier, VT  
05620-3402

STATE OF VERMONT  
SECRETARY OF STATE

By: 

Rachel J. Allen  
State Prosecuting Attorney  
802-828-1217